

BLUE REPORT

Rituximab maintenance therapy is optional following complete response to rituximab monotherapy in splenic marginal zone lymphoma. According to the National Comprehensive Cancer Network (NCCN) guidelines, rituximab maintenance at 375 mg/m² every 8-12 weeks for up to 2 years is listed as **optional first-line extended therapy** following complete response (CR) or partial response (PR) for patients treated with single-agent rituximab.[\[1\]](#)

The evidence supporting rituximab maintenance in marginal zone lymphoma is limited and somewhat controversial. A NEJM review notes that "the need for maintenance treatment with rituximab in patients with MZLs is controversial, with no evidence of a survival benefit." While some studies have shown improved progression-free survival with maintenance therapy, no overall survival benefit has been demonstrated.[\[2\]](#)

Key considerations for your patient:

Evidence for maintenance therapy:

- A phase II study of R-CVP followed by rituximab maintenance in advanced MZL showed a 3-year PFS of 81% and 3-year OS of 90%, with maintenance therapy being well-tolerated.[\[3\]](#)
- One retrospective series in splenic MZL reported that maintenance rituximab improved CR rates (from 42% to 71%) and 7-year PFS (75% vs 39%, $p < 0.0004$), though no OS difference was observed.[\[4\]](#)
- A phase III trial (MabCute) evaluating extended maintenance beyond 2 years in relapsed/refractory indolent NHL found insufficient evidence to support prolonged maintenance beyond the standard 2-year period.[\[5\]](#)

Arguments against routine maintenance:

- No demonstrated overall survival benefit in MZL specifically
- Your patient achieved a complete metabolic response, which is associated with favorable outcomes
- She is 82 years old with comorbidities (spina bifida, asthma), making treatment-related toxicity a consideration
- Active surveillance is an acceptable alternative per NCCN guidelines[\[1\]](#)

Reasonable approaches:

1. **Active surveillance** with clinical follow-up every 3-6 months for 5 years, then annually, with imaging as clinically indicated

2. Optional rituximab maintenance (375 mg/m² every 8-12 weeks for up to 2 years) if you and the patient value potentially prolonged PFS despite lack of OS benefit

Given her age, complete response, and the lack of survival benefit from maintenance therapy, active surveillance is a very reasonable approach. However, if she and her family place high value on potentially extending the time to progression, maintenance therapy remains an option.

Would you like me to explore the specific surveillance imaging recommendations and monitoring parameters for patients with splenic marginal zone lymphoma in complete remission?

References

1. [B-Cell Lymphomas](#). National Comprehensive Cancer Network. Updated 2025-12-22.
2. [Marginal-Zone Lymphomas](#). Rossi D, Bertoni F, Zucca E. The New England Journal of Medicine. 2022;386(6):568-581. doi:10.1056/NEJMra2102568.
3. [Phase II Study of R-CVP Followed by Rituximab Maintenance Therapy for Patients With Advanced Marginal Zone Lymphoma: Consortium for Improving Survival of Lymphoma \(CISL\) Study](#). Oh SY, Kim WS, Kim JS, et al. Cancer Communications (London, England). 2019;39(1):58. doi:10.1186/s40880-019-0403-7.
4. [Should Rituximab Replace Splenectomy in the Management of Splenic Marginal Zone Lymphoma?](#) Kalpadakis C, Pangalis GA, Angelopoulou MK, Sachanas S, Vassilakopoulos TP. Best Practice & Research. Clinical Haematology. 2018;31(1):65-72. doi:10.1016/j.beha.2017.10.011.
5. [Efficacy and Safety Assessment of Prolonged Maintenance With Subcutaneous Rituximab in Patients With Relapsed or Refractory Indolent Non-Hodgkin Lymphoma: Results of the Phase III MabCute Study](#). Rule S, Barreto WG, Briones J, et al. Haematologica. 2022;107(2):500-509. doi:10.3324/haematol.2020.274803.